Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
7703021000, 7897319900
www.purepathlab.in Gmail: help@purepathlab.in
PURE PATH LAB
Palce of Accuracy & Your Satisfaction
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

Mrs. Shanti Prasad
Age / Sex : 60 YRS / F
Reg. no. : 7016

Registered on : -
Collected on : -
Received on : -
Reported on : -

HAEMATOLOGY

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**L**	**10.3**	**g/dl**	**12 - 15**
TOTAL LEUKOCYTE COUNT		6,700	cumm	4,800 - 10,800
NEUTROPHILS		70	%	40 - 80
LYMPHOCYTE	**L**	**16**	**%**	**20 - 40**
EOSINOPHILS	**H**	**12**	**%**	**1-6**
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	<2
PLATELET COUNT		2.91	lakhs/cumm	1.5 - 4.1
TOTAL RBC COUNT	**L**	**3.80**	**million/cumm**	**3.9 - 4.8**
HEMATOCRIT VALUE, HCT	**L**	**26.5**	**%**	**36 - 46**
MEAN CORPUSCULAR VOLUME, MCV	**L**	**69.7**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH		27.1	Pg	27 - 32
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**38.9**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		9.6	fL	6.5 - 12
R.D.W. - SD		44.9	fL	39 - 46
R.D.W. - CV	**H**	**18.7**	**%**	**11.6 - 14**
P-LCR		26.1	19.7	- 42.4
P.D.W.	**H**	**15.6**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

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SEROLOGY & IMMUNOLOGY

TEST	VALUE	UNIT	REFERENCE
C-REACTIVE PROTEIN, CRP (QUALITATIVE)	0.89	mg/L	< 6 mg/L

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BIOCHEMISTRY

TEST		VALUE	UNIT	REFERENCE
SERUM UREA	**H**	**47.51**	**mg/dl**	**21 - 43**
SERUM CREATININE		1.06	mg/dl	0.55 - 1.13
SERUM PHOSPHORUS		4.44	mg/dl	2.5 - 4.5
SERUM URIC ACID	**H**	**6.57**	**mg/dl**	**2.6 - 6**
SERUM SODIUM		137.5	mmol/L	136 - 146
SERUM POTASSIUM		4.66	mmol/L	3.5 - 5.1
ICALCIUM		1.13	mmol/l	1.13 - 1.33

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

SERUM BILIRUBIN (TOTAL)		0.96	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)		0.17	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)		0.79	mg/dl	0.2 - 1
SGOT (AST)		26.87	U/l	0 - 31
SGPT (ALT)		28.96	U/l	7 - 32
SERUM ALKALINE PHOSPHATASE		93.56	U/l	44 - 147
SERUM PROTEIN	**L**	**6.01**	**g/dl**	**6.4 - 8.3**
SERUM ALBUMIN	**L**	**3.44**	**g/dl**	**3.5 - 5.2**
GLOBULIN		2.57	g/dl	1.8 - 3.6
A/G RATIO		1.34	1.1	- 2.1

LFT Interpretation
Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance
Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.
Increased in: Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

RANDOM BLOOD SUGAR	101.6	mg/dl	70 - 140

~~~ End of report ~~~
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